

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

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Finding reasoning paths :

- 1. Hyper activity and lack of concentration -> Impulsivity and inattention -> Hyperactive - impulsive behavior -> Autism spectrum disorder (ASD).
- 2. Preschool expulsion and difficulty with concentration -> Impulsivity and inattention -> Social and behavioral issues -> ASD .
- 3. Otitis media history -> Hearing impairment -> Impaired listening skills -> ASD .
- 4. Major depressive disorder in the mother -> Genetic or environmental predisposition -> ASD .
- 5. Graves ' disease in the father -> Thyroid dysfunction -> Cognitive or behavioral abnormalities -> ASD .

Reasoning Process :

- 1. **Symptoms Analysis**: The patient exhibits symptoms of hyperactivity , impulsivity , inattention , and disruptive behavior , which are commonly associated with Autism Spectrum Disorder (ASD). These behaviors can lead to difficulties in social interactions , communication , and adherence to rules , which are hall marks of ASD .
- 2. **Behavioral History**: The patient 's history of being expelled from preschool for disruptive behavior , along with difficulty concentrating and impulsivity , aligns with characteristics of ASD . These behavioral issues often arise from challenges in managing attention and following social norms , which are central to ASD .
- 3. **Developmental Milestones**: Despite meeting all developmental milestones , the patient 's behavioral issues may indicate that he is struggling with specific developmental aspects such as social communication , which are central to ASD diagnosis .
- 4. **Family History**: The mother 's major depressive disorder and the father 's Graves ' disease do not directly link to ASD but could contribute to environmental or genetic factors that predispose the child to ASD . However , these conditions are not primary indications for ASD .
- 5. **Otitis Media History**: While otitis media can lead to temporary hearing loss and affect communication and listening skills , the current symptoms and behavior described are more indicative of ASD rather than hearing -related issues .

Conclusion :

Based on the symptoms , behavioral history , and developmental considerations , the patient 's presentation is most consistent with Autism Spectrum Disorder . Therefore , the most appropriate next step in treatment would be early intervention and behavioral therapy to address the behavioral and communication challenges associated with ASD .

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<answer>

Behavior therapy

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